

GASTRITIS

Inflammation of the stomach lining, called gastritis, causes discomfort or pain, as well as nausea and vomiting. Gastritis that comes on suddenly, known as sudden onset (acute) gastritis, may be caused by over-indulging, especially in alcohol consumption, or by taking medications known for their effect on the stomach lining, such as aspirin. Chronic gastritis develops over the longer term and may be due to repeated insult to the lining by alcohol, tobacco, or drugs. Another common cause is the bacterium *Helicobacter pylori*. Gastritis usually gets better with medication and by removing the underlying cause.



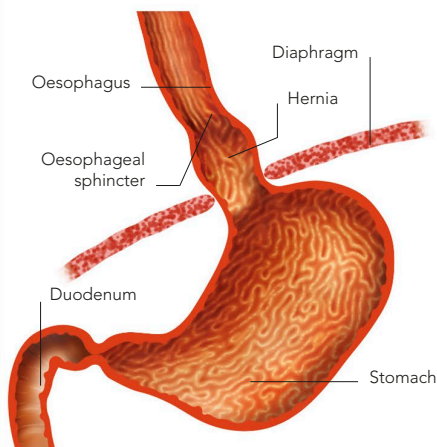
COMMON CULPRIT

At least 50 per cent of people have *H. pylori* in their stomach lining. If the bacteria cause symptoms, antibiotics can eradicate them.

HIATUS HERNIA

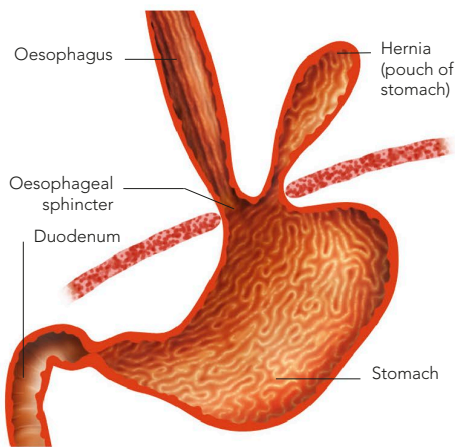
The oesophagus passes through a taut gap (hiatus) in the muscular sheet of the diaphragm, which lies between the abdomen and the chest cavity. The hiatus helps the oesophageal sphincter (ring of muscle at the lower end of the oesophagus) to prevent acidic stomach contents from passing up into the lower oesophagus. In a hiatus hernia, the upper section of the stomach protrudes up

through this gap. Any symptoms of a hiatus hernia are those of heartburn (gastric reflux). There are two types of hiatus hernia: sliding and para-oesophageal. Sliding hernias usually have no symptoms, and it is estimated that they are present in around a third of all people over 50. In rare cases, however, para-oesophageal hernias can cause severe pain and require surgery.



SLIDING HIATUS HERNIA

This is the most common type of hiatus hernia, and occurs when the junction between the oesophagus and the stomach slides up through the diaphragm.



PARA-OESOPHAGEAL HIATUS HERNIA

In about 1 in 10 hernias, a pouch-like part of the stomach is pushed upward through the diaphragm and lies adjacent to the lower oesophagus.